Common Childhood Conditions

Comprehensive Guide: Symptoms, Home Treatment, and When to See a Doctor

Covering: Bronchiolitis, Cough, Diaper Rash, Teething, Cradle Cap, Nosebleeds, Ringworm, Influenza, and UTIs

Based on MedlinePlus/NIH, AAP, CDC, and major pediatric hospitals. For educational purposes only.

1. Bronchiolitis

Bronchiolitis is a common lung infection in infants and young children under 2. It causes swelling and mucus buildup in the small airways (bronchioles), making breathing difficult. RSV (respiratory syncytial virus) is the most common cause — more than half of all infants are exposed to RSV by their first birthday. Other causes include adenovirus, influenza, parainfluenza, and human metapneumovirus. Peak incidence is between 3 and 6 months of age. It occurs more often in fall and winter and is a very common reason for infant hospitalization during those months.

Risk Factors

- Being around cigarette smoke
- Being younger than 6 months old
- Living in crowded conditions
- Not being breastfed
- Being born before 37 weeks of pregnancy (premature)
- Having heart or lung conditions, or a weakened immune system

Symptoms

Bronchiolitis begins as a mild upper respiratory infection. Within 2–3 days, the child develops more breathing problems:

- **Early (days 1–3):** Runny nose, congestion, mild cough, possible low-grade fever
- **Progressing (days 2–5):** Wheezing, worsening cough, faster breathing, decreased appetite, fatigue
- **Severe signs:** Bluish skin color due to lack of oxygen (cyanosis) — emergency treatment needed. Muscles around ribs sink in with each breath (intercostal retractions). Nostrils flare when breathing. Rapid breathing (tachypnea). Difficulty feeding (cannot suck and breathe at the same time).

How It Spreads

The virus spreads through direct contact with nose and throat fluids — when someone nearby sneezes or coughs and droplets are breathed in, or when an infant touches contaminated toys or surfaces. A child is contagious for approximately 3–8 days. The virus can survive on surfaces for several hours.

Home Treatment

HOME CARE: Bronchiolitis is a virus — antibiotics do NOT help. Treatment is supportive care.

- **Fluids:** Have your child drink plenty of fluids. Breast milk or formula for children under 12 months. Electrolyte drinks such as Pedialyte are also OK for infants. Offer smaller, more frequent feedings since breathing takes extra effort.
- **Humidifier:** Use a cool-mist humidifier to moisten the air and help loosen sticky mucus. Clean it daily.
- **Nasal suctioning:** Give saline nose drops (2–3 drops per nostril), then use a bulb syringe or NoseFrida to clear mucus. Do this before feedings and sleep. This is the single most helpful home treatment.
- **Rest:** Make sure your child gets plenty of rest.
- **Fever:** Acetaminophen for infants 2 months+. Ibuprofen for 6 months+ only. Contact your pediatrician for dosing. Never give aspirin.
- **No smoking:** Do not allow anyone to smoke in the house, car, or anywhere near your child.
- **Do NOT give:** Over-the-counter cough or cold medicines (not safe and not effective for bronchiolitis). Honey to infants under 12 months.

Timeline and Outlook

Breathing often gets better by the third day, and symptoms mostly clear within a week. In rare cases, pneumonia or more severe breathing problems develop. Some children may have problems with wheezing or asthma as they get older. About 2–3% of infants with bronchiolitis require hospitalization for oxygen therapy and/or IV fluids.

Prevention

- Careful hand washing, especially around infants
- Keep infants away from people who are sick
- Avoid crowded indoor spaces during RSV season
- Breastfeed if possible (provides protective antibodies)
- **Palivizumab (Synagis):** A medicine that boosts the immune system, may be recommended for certain high-risk children (premature infants, those with heart or lung conditions). Your pediatrician will advise if this is appropriate for your child. RSV maternal vaccination during pregnancy is also now available.

SEE A DOCTOR IF: Baby becomes extremely tired or difficult to wake. Bluish color in skin, nails, or lips. Starts breathing very fast. Has a cold that suddenly worsens. Has difficulty breathing, nostril flaring, or chest retractions. Is under 12 weeks with any fever (100.4°F+). Refuses to eat or drink for 2+ consecutive feedings. Fewer wet diapers than normal (dehydration). Any premature baby or baby with heart/lung conditions showing symptoms.

Sources: MedlinePlus/NIH; AAP HealthyChildren.org; Mayo Clinic; StatPearls

2. Cough in Children

Coughing is an important way to keep the throat and airways clear. It is one of the most common reasons parents bring children to the doctor. Most coughs are caused by viral upper respiratory infections (colds) and resolve on their own. Coughs can be acute (under 3 weeks, usually from a cold or flu), subacute (3–8 weeks), or chronic (longer than 8 weeks). The character of the cough can help identify the cause.

Types of Cough and What They Mean

- **Dry, tickling cough:** Common with colds, allergies, or asthma. No mucus. Often worse at night.
- **Wet, productive cough:** Produces mucus. Common with colds, sinus infections, or lower respiratory infections.
- **Barking cough (like a seal):** Characteristic of croup. Caused by swelling of the upper airway. Often worse at night. May have stridor (high-pitched noise breathing in).
- **Whooping cough:** Rapid coughing fits followed by "whoop" sound when inhaling. Characteristic of pertussis. Can cause vomiting after fits. Vaccine-preventable. Dangerous in infants.
- **Wheezing cough:** Whistling sound breathing out. Suggests narrowed lower airways. Common in bronchiolitis (infants) and asthma (older children).
- **Night cough:** Worse at night or lying down. Common with post-nasal drip (colds/allergies), asthma, and gastroesophageal reflux (GERD).
- **Cough with exercise:** May indicate exercise-induced asthma.
- **Violent cough that begins rapidly:** Could indicate foreign body aspiration (something inhaled into the airway) — this is an emergency if the child suddenly starts coughing violently with no preceding cold symptoms.

Causes

Common causes in children: common cold, flu, and other viral infections; allergies (with post-nasal drip); asthma; sinus infections; pneumonia and bronchitis; croup (viral laryngotracheobronchitis); gastroesophageal reflux disease (GERD — stomach acid irritating the throat, especially at night); exposure to secondhand smoke or other irritants; and foreign body in the airway.

Home Treatment

HOME CARE: Most coughs from colds resolve in 1–2 weeks. The goal is comfort — coughing helps clear the airways.

- **Under 1 year:** Breast milk or formula for hydration. Saline drops + nasal suctioning. Cool-mist humidifier. NO honey. NO cough medicine.
- **Ages 1–5:** Honey: 1/2 to 1 teaspoon (2–5 mL) as needed — evidence-based cough remedy. Warm fluids (warm water with honey, warm apple juice). Humidifier.
- **Ages 6+:** Honey or hard candy/cough drops for throat soothing. Warm fluids. Humidifier. Elevate head of bed slightly. For older children, guaifenesin may help break up mucus (follow package instructions and drink lots of fluids).
- **Choking hazard:** Never give cough drops or hard candy to a child under age 3.
- **All ages:** Keep child hydrated. Avoid irritants (cigarette smoke, strong fragrances). Cool-mist humidifier. For allergies: stay indoors during high allergen times, keep windows closed, shower and change clothes after being outside.
- **For barking cough (croup):** Take the child into a steamy bathroom (hot shower running, door closed) for 10–15 minutes. OR take outside into cool night air briefly. Both reduce airway swelling. If stridor at rest, call pediatrician or go to ER.

• **Do NOT give:** Over-the-counter cough and cold medicines to children under 6 years (per MedlinePlus/AAP/FDA). These are not proven effective and can cause serious side effects. Talk to your provider before giving any OTC cough medicine to children under 6. Codeine-containing cough medicines are not recommended for any child.

SEE A DOCTOR IF: Cough lasts more than 10–14 days (contact provider). Difficulty breathing (fast breathing, retractions, nasal flaring). Cough produces blood. Thick, foul-smelling, yellowish-green phlegm (may indicate bacterial infection). High-pitched stridor when breathing in. High fever with cough. Child cannot eat, drink, or sleep due to coughing. Infant under 3 months with any cough. Whooping sound after cough fits. Violent cough that began rapidly (possible foreign object — emergency). Cough gets worse when lying down with heart or swelling problems.

Sources: MedlinePlus/NIH; AAP HealthyChildren.org; Mayo Clinic

3. Diaper Rash

Diaper rash (diaper dermatitis) is one of the most common skin conditions in infants/toddlers. It appears as red, irritated skin in the diaper area. Most cases are caused by prolonged contact with wet or soiled diapers. Nearly every baby gets diaper rash at some point — it is not a sign of neglect. Stool is more irritating than urine, so rash is often worse during episodes of diarrhea.

Types

• **Irritant contact dermatitis (most common):** Red, shiny patches on buttocks and skin folds from prolonged moisture exposure.

• **Yeast (Candida) diaper rash:** Bright red rash with well-defined edges and small red satellite spots around the border. Often in skin folds. Common after antibiotic use. Does not improve with barrier cream alone — needs antifungal.

• **Allergic contact dermatitis:** Reaction to wipes, diaper brand, detergent, or cream. Appears where product contacts skin.

• **Bacterial infection (rare):** Bright red, warm, tender skin, sometimes with blisters or pus-filled bumps. May have fever.

Home Treatment

HOME CARE: Most diaper rash clears within 3–4 days. Key: keep the area clean, dry, and protected.

• **Change diapers frequently:** Every 1–2 hours, or immediately after bowel movement. Most effective step.

• **Clean gently:** Warm water + soft cloth. Fragrance-free, alcohol-free wipes if using wipes. Pat dry, do not rub. Let air-dry completely before new diaper. Avoid tight-fitting diapers.

• **Barrier cream:** Thick layer of zinc oxide (Desitin, A+D, Boudreaux's Butt Paste) at every change. Apply like frosting. Don't wipe off completely each time.

• **Diaper-free time:** 10–15 minutes several times daily on a waterproof pad. Air exposure is very effective.

• **For yeast rash:** OTC antifungal cream (clotrimazole or miconazole) at every change, with barrier cream on top.

• **Warm baths:** 1–2 times daily in plain water. No bubble bath.

• **Avoid:** Baby powder (inhalation risk), cornstarch (feeds yeast), perfumed products.

SEE A DOCTOR IF: Rash doesn’t improve after 3–4 days of home treatment. Blisters, open sores, pus-filled bumps, or weeping. Bright red with satellite spots (likely yeast). Baby has fever with the rash. Rash spreads beyond diaper area. Significant pain during diaper changes.

Sources: MedlinePlus/NIH; AAP HealthyChildren.org; Mayo Clinic

4. Teething

Teething is the growth of teeth through the gums. It generally begins between 6 and 8 months, though some children start earlier or later. All 20 baby (primary) teeth should be in place by about 30 months. Later teething is usually normal and not due to disease.

Tooth Eruption Order

- Two bottom front teeth (lower central incisors) usually come in first
- Then two top front teeth (upper central incisors)
- Then other incisors, lower and upper molars, canines, and finally the upper and lower lateral molars

Symptoms

- **Irritability/crankiness:** Especially in the days before a tooth breaks through
- **Biting/chewing on hard objects:** To relieve gum pressure
- **Drooling:** Often significant, may begin before teething starts. Can cause a mild chin/mouth rash
- **Gum swelling and tenderness:** Area may appear red or have a visible white bump
- **Refusing food**
- **Sleeping problems**
- **Ear pulling/cheek rubbing:** Shared nerve pathways between gums, ears, and cheeks. But persistent ear pulling WITH fever may be an ear infection, not teething.

What teething does NOT cause: Fever or diarrhea. If your child develops a fever or diarrhea, contact your health care provider — these indicate illness, not teething. The timing of teething (6–12 months) coincides with declining maternal antibodies, so babies often get sick around the same time.

Home Treatment

HOME CARE: Teething is normal and temporary. The goal is comfort.

- **Cool teething ring or washcloth:** Chill (do not freeze) a firm rubber teething ring or wet washcloth. Avoid liquid-filled teething rings or plastic objects that might break. Do not place anything frozen against the gums.
- **Gum massage:** Gently rub gums with a cool, wet washcloth or clean finger until teeth are near the surface.
- **Cool, soft foods (if on solids):** Chilled applesauce, cold yogurt, frozen fruit in a mesh feeder.
- **Drool management:** Wipe face with cloth to prevent rash. Petroleum jelly on chin/cheeks to protect skin.
- **Pain relief:** Acetaminophen or ibuprofen (6 months+) for significant discomfort. Contact your pediatrician for dosing.
- **Bottle:** If a bottle seems to comfort, only fill it with water. Formula, milk, or juice can cause tooth decay.

What NOT to Do

- **Do not use benzocaine gels** (Orajel, Anbesol) — FDA warns against use in children under 2 due to risk of methemoglobinemia, a serious blood condition.
- **Do not use homeopathic teething tablets or powders** — some contain inconsistent and potentially toxic amounts of belladonna.
- **Avoid teething powders** in general.
- **Do not tie a teething ring or any object around your child's neck** — strangulation hazard. This includes amber teething necklaces, which have no proven benefit and pose strangulation and choking risks.
- **Never cut the gums** to help a tooth grow in — this can lead to infection.
- **Never give aspirin** or place it against gums.

- **Do not rub alcohol** on the gums — even small amounts are harmful to a baby.

SEE A DOCTOR IF: True fever of 100.4°F (38°C) or higher (not caused by teething — child is sick). Diarrhea, vomiting, or widespread rash (not caused by teething). Child refuses to eat or drink for an extended period. No teeth by 18 months (uncommon but should be evaluated).

Sources: MedlinePlus/NIH; AAP HealthyChildren.org; FDA Benzocaine Safety Warning

5. Cradle Cap (Seborrheic Dermatitis)

Cradle cap is a very common, harmless skin condition in newborns and young infants. It is a form of seborrheic dermatitis caused by a combination of oil gland activity, a yeast called Malassezia that lives naturally on the skin, changes in skin barrier function, and genetics. It is NOT caused by poor hygiene, allergies, or infection. It is not contagious and does not bother the baby. Most cases resolve on their own by 6–12 months.

Appearance

- Thick, yellowish or white crusty or scaly patches on the scalp
- May also appear on eyebrows, eyelids, creases of the nose, lips, behind the ears, or in the outer ear
- Skin underneath may be slightly red
- Greasy or oily-looking skin scales
- Not itchy or painful (unlike eczema)

Home Treatment

HOME CARE: Cradle cap is cosmetic and does not require treatment. If desired:

- **Oil and brush method:** Apply mineral oil, coconut oil, or petroleum jelly to scaly areas. Wait 15–20 minutes to soften. Gently brush with a soft baby brush or fine-toothed comb to lift flakes. Wash with mild baby shampoo afterward. Do not pick at scales with fingernails.
- **Regular shampooing:** Mild baby shampoo 2–3 times per week. Gently massage scalp with fingertips.
- **Medicated shampoo (stubborn cases):** Pediatrician may recommend a mild dandruff shampoo with selenium sulfide or ketoconazole, used 1–2 times per week. Leave on 2 minutes, rinse. Keep out of eyes. Do not use adult dandruff shampoo without asking your pediatrician.

SEE A DOCTOR IF: Scales spread to face, body, or diaper area (may indicate broader seborrheic dermatitis or eczema). Area becomes red, swollen, warm, or starts weeping/oozing (possible infection). Does not improve after several weeks. Baby seems itchy or uncomfortable (may not be cradle cap). Drainage of fluid or pus, or scales become very red or painful.

Sources: MedlinePlus/NIH; AAP HealthyChildren.org

6. Nosebleeds (Epistaxis)

Nosebleeds are very common in children ages 2–10 and are rarely serious. Most are anterior (from the front of the nose) involving small blood vessels in the nasal septum. Common triggers: dry air, nose picking, colds/allergies (inflamed nasal membranes), or minor trauma. They look scary but are almost always harmless.

Home Treatment (Correct Technique)

HOME CARE: Most nosebleeds stop within 5–10 minutes with proper technique:

- **Step 1 — Stay calm.** Reassure the child. Nosebleeds are rarely dangerous.

- **Step 2 — Sit upright and lean slightly FORWARD.** Do NOT tilt head back — blood flows down the throat causing gagging/vomiting.
- **Step 3 — Pinch the soft part of the nose** (below the bony bridge) firmly. Press both nostrils closed. Hold for a full **10 minutes without checking.** Time it. Checking too early restarts bleeding.
- **Step 4 — Cold compress** on the bridge of the nose while pinching.
- **Step 5 — If still bleeding after 10 minutes,** repeat for another 10 minutes.
- **After it stops:** No blowing nose, picking, or bending over for several hours. Apply petroleum jelly inside nostrils.

Prevention

- Humidifier during dry weather/winter
- Petroleum jelly inside nostrils daily during dry months
- Keep fingernails trimmed
- Treat allergies/colds to reduce nasal inflammation
- Saline nasal spray to keep membranes moist

SEE A DOCTOR IF: Bleeding does not stop after 20 minutes of firm pressure. Nosebleeds very frequent (more than once a week). Followed a head injury or blow to the face. Child is dizzy, pale, or faint. Large amount of blood. Child under 2 (less common, worth evaluating). Easy bruising elsewhere or family history of bleeding disorders.

Sources: MedlinePlus/NIH; AAP; Mayo Clinic

7. Ringworm (Tinea)

Despite its name, ringworm is NOT caused by a worm. It is a common fungal skin infection (dermatophytes) creating a red, ring-shaped, scaly rash. Very common in children, contagious but easily treatable.

Types by Location

- **Tinea corporis (body):** Red, circular, scaly patches with clearer center. Itchy. Arms, legs, trunk.
- **Tinea capitis (scalp):** Scaly, itchy patches causing hair loss. Requires oral antifungal (topical alone doesn't penetrate hair follicle). More common in children.
- **Tinea pedis (athlete's foot):** Itchy, cracked skin between toes. More common in teens.
- **Tinea cruris (jock itch):** Red, itchy rash in groin folds. More common in adolescents.

How It Spreads

Direct skin-to-skin contact with infected person, animal (cats/dogs carry it), or contaminated objects (towels, combs, hats, sports equipment, floors). Thrives in warm, moist environments. Contagious until 48 hours after starting treatment.

Home Treatment (Body Ringworm)

- **Antifungal cream:** OTC clotrimazole (Lotrimin), miconazole, or terbinafine (Lamisil). Apply to rash AND 1–2 cm surrounding skin, twice daily. Continue **1–2 weeks after rash appears clear** (total 2–4 weeks). Stopping early causes recurrence.
- Keep area clean and dry
- Avoid sharing towels, combs, brushes, hats, clothing
- Wash bedding and clothing in hot water
- Check pets for patchy hair loss or scaly skin — common reinfection source
- Child can attend school/daycare after starting treatment if rash is covered

SEE A DOCTOR IF: Rash on the scalp (requires prescription oral antifungal). Not improving after 2 weeks of OTC treatment. Spreading despite treatment. Red, swollen, warm, or oozing pus (secondary bacterial infection). Child has weakened immune system.

Sources: MedlinePlus/NIH; AAP; CDC Fungal Diseases

8. Influenza (Flu) in Children

Influenza is a contagious respiratory illness caused by influenza viruses — not the same as "stomach flu" (gastroenteritis). It affects the nose, throat, and lungs. Children under 5, especially under 2, are at higher risk for serious complications including pneumonia, dehydration, and rarely, death. Flu season: October–May, peaking December–February.

Symptoms (Come On Suddenly)

- **Fever** (often 101–104°F), lasting 3–5 days
- **Body aches and headache** ("everything hurts")
- **Chills and extreme fatigue**
- **Dry, persistent cough**
- **Sore throat and runny nose**
- **Vomiting and diarrhea** (more common in children than adults)

- **Decreased appetite**

Flu vs. cold: Flu hits hard and fast (high fever, body aches, exhaustion within hours). A cold develops gradually (runny nose, mild cough, low/no fever, child still somewhat active).

Home Treatment

HOME CARE: Most healthy children recover within 1–2 weeks.

- **Rest:** Let the child sleep as much as they want.
- **Fluids:** Frequent small sips — water, clear broth, Pedialyte, diluted juice, popsicles. Dehydration is the most common home complication.
- **Fever and pain:** Acetaminophen or ibuprofen (6 months+). **NEVER aspirin** — risk of Reye's syndrome.
- **Humidifier + saline:** For congestion.
- **Honey (1+ year):** 1/2 to 1 teaspoon for cough.
- **Tamiflu (oseltamivir):** Prescription antiviral. Reduces severity by 1–2 days IF started within 48 hours. Recommended for high-risk children (under 2, asthma, diabetes, immune-compromised). Can be given to infants as young as 2 weeks. Call your pediatrician within the first 1–2 days of symptoms.
- **Prevention:** Annual flu vaccine for everyone 6 months+. Best protection. Takes ~2 weeks to become fully effective.

SEE A DOCTOR IF: Difficulty breathing or fast breathing. Blue/gray lips. Ribs pulling in with breaths. Severe or persistent vomiting. Not drinking / dehydration signs. Not waking up, not interactive, or extremely irritable. Symptoms improve then return with fever and worse cough (possible secondary pneumonia). Fever above 104°F. Any child under 2 with flu symptoms (call pediatrician — Tamiflu may be recommended). Child with chronic conditions (asthma, diabetes, heart disease, immune suppression).

Sources: MedlinePlus/NIH; CDC Influenza; AAP HealthyChildren.org

9. Urinary Tract Infections (UTIs) in Children

A UTI is a bacterial infection in any part of the urinary system (kidneys, ureters, bladder, urethra). Escherichia coli (E. coli) causes approximately 85% of UTIs in children. UTIs are the second most common bacterial infection in children (after ear infections). About 8% of girls and 2% of boys have at least one UTI by age 7. UTIs are more common in girls (shorter urethra) and in uncircumcised boys under 1 year. UTIs always require antibiotics — they do not resolve on their own and can cause kidney damage if untreated.

Risk Factors

- Female sex (shorter urethra, closer to anus)
- Uncircumcised males under 1 year (10x higher risk than circumcised boys)
- Wiping from back to front (spreads bacteria from bowel to urethra)
- Not urinating often enough / "holding it"
- Constipation (full rectum presses on bladder, prevents complete emptying) — major risk factor for recurrent UTIs
- Vesicoureteral reflux (VUR): urine flows backwards from bladder to kidneys. Found in 8–40% of children evaluated for first UTI.
- Structural abnormalities of the urinary tract
- Bubble baths and irritants
- Infrequent diaper changes in infants

Symptoms by Age

UTI symptoms vary significantly by age, making diagnosis tricky in infants:

- **Infants (under 2 years):** Fever may be the ONLY sign (fever with no obvious source in a baby should prompt UTI testing). Other signs: irritability, poor feeding, vomiting, foul-smelling urine, failure to gain weight, lethargy, jaundice in newborns.
- **Toddlers/preschoolers:** Fever, pain or burning with urination (may cry or resist going), frequent urination or urgency, bedwetting in a previously dry child, abdominal or lower back pain, foul-smelling or cloudy urine, blood in urine (pink/red).
- **School-age and teens:** Burning/stinging during urination, frequent urgent need (sometimes only small amounts), lower abdominal pressure, blood/cloudy/foul-smelling urine. **Kidney infection (pyelonephritis) symptoms:** high fever, back or side (flank) pain, nausea/vomiting, feeling very ill.

Treatment

UTIs require antibiotics prescribed by a doctor. Prompt treatment protects the kidneys.

- **See your pediatrician promptly** for urine testing. Diagnosis requires a urine sample (clean catch for older children, catheterization for infants).
- **Any child under 6 months** with a UTI should see a specialist right away.
- **Younger infants** will most often need to stay in the hospital and receive antibiotics through a vein (IV).
- **Older infants and children:** Oral antibiotics (commonly cephalexin, trimethoprim-sulfamethoxazole, or amoxicillin-clavulanate) for 7–14 days. Complete the full course even if symptoms improve quickly.
- **Fluids:** Encourage plenty of water to flush bacteria.
- **Pain relief:** Acetaminophen or ibuprofen. Warm compress on lower abdomen for comfort.
- **Follow-up imaging:** Children with UTIs before age 2, or with recurrent UTIs, may need ultrasound or other imaging to check for structural abnormalities (such as vesicoureteral reflux).

- **Recurrent UTIs:** Some children may need long-term low-dose antibiotics (6 months to 2 years) to prevent recurrence, especially if vesicoureteral reflux is present.

Prevention

- **Wiping direction (girls):** Always front to back
- **Hydration:** Regular water intake throughout the day
- **Regular bathroom breaks:** Every 2–3 hours, do not hold it
- **Avoid irritants:** No bubble baths, no tight underwear, wear cotton underwear
- **Treat constipation:** Chronic constipation is a major UTI risk factor
- **Frequent diaper changes** in babies and toddlers
- **Proper hygiene:** For uncircumcised boys, do not forcibly retract the foreskin

SEE A DOCTOR IF: Any infant with unexplained fever (100.4°F+), especially under 3 months. Burning or pain with urination. Blood, cloudiness, or foul smell in urine. Symptoms return after treatment or recur (2+ in 6 months or 3+ in a year — may need imaging). Back or side pain with fever (possible kidney infection — urgent). Child looks very ill, high fever, or vomiting and unable to keep down fluids/oral antibiotics (may need IV antibiotics in hospital).

Sources: MedlinePlus/NIH; AAP UTI Clinical Practice Guidelines; Mayo Clinic; StatPearls; AAFP
